

AMENDMENT OF SOLICITATION/MODIFICATION OF CONTRACT		BPA NO.	1. CONTRACT ID CODE	PAGE	OF	PAGES
2. AMENDMENT/MODIFICATION NUMBER 0003		3. EFFECTIVE DATE 04-29-2026		4. REQUISITION/PURCHASE REQ. NUMBER		
5. PROJECT NUMBER (if applicable) MSO-2026-COM-0013						
6. ISSUED BY Department of Veterans Affairs Network Contracting Office (NCO) 21		7. ADMINISTERED BY (If other than Item 6) Department of Veterans Affairs Network Contracting Office (NCO) 21		CODE		
8. NAME AND ADDRESS OF CONTRACTOR (Number, street, county, State and ZIP Code) To all Offerors/Bidders		(X)		9A. AMENDMENT OF SOLICITATION NUMBER 36C26126R0014		
		☒		9B. DATED (SEE ITEM 11)		
		☐		10A. MODIFICATION OF CONTRACT/ORDER NUMBER		
				10B. DATED (SEE ITEM 13)		
CODE		FACILITY CODE				
11. THIS ITEM ONLY APPLIES TO AMENDMENTS OF SOLICITATIONS						
☒ The above numbered solicitation is amended as set forth in Item 14. The hour and date specified for receipt of Offers ☒ is extended, ☐ is not extended. Offers must acknowledge receipt of this amendment prior to the hour and date specified in the solicitation or as amended, by one of the following methods: (a) By completing Items 8 and 15, and returning <u>1</u> copies of the amendment; (b) By acknowledging receipt of this amendment on each copy of the offer submitted; or (c) By separate letter or electronic communication which includes a reference to the solicitation and amendment numbers. FAILURE OF YOUR ACKNOWLEDGMENT TO BE RECEIVED AT THE PLACE DESIGNATED FOR THE RECEIPT OF OFFERS PRIOR TO THE HOUR AND DATE SPECIFIED MAY RESULT IN REJECTION OF YOUR OFFER. If by virtue of this amendment you desire to change an offer already submitted, such change may be made by letter or electronic communication, provided each letter or electronic communication makes reference to the solicitation and this amendment, and is received prior to the opening hour and date specified. ** HOUR & DATE for Receipt of Offers is EXTENDED to: 05-15-2026 10AM PDT						
12. ACCOUNTING AND APPROPRIATION DATA (If required)						
13. THIS ITEM APPLIES ONLY TO MODIFICATIONS OF CONTRACTS/ORDERS, IT MODIFIES THE CONTRACT/ORDER NO. AS DESCRIBED IN ITEM 14.						
CHECK ONE	A. THIS CHANGE ORDER IS ISSUED PURSUANT TO: (Specify authority) THE CHANGES SET FORTH IN ITEM 14 ARE MADE IN THE CONTRACT ORDER NO. IN ITEM 10A.					
☐	B. THE ABOVE NUMBERED CONTRACT/ORDER IS MODIFIED TO REFLECT THE ADMINISTRATIVE CHANGES (such as changes in paying office, appropriation date, etc.) SET FORTH IN ITEM 14, PURSUANT TO THE AUTHORITY OF FAR 43.103(b).					
☐	C. THIS SUPPLEMENTAL AGREEMENT IS ENTERED INTO PURSUANT TO AUTHORITY OF:					
☐	D. OTHER (Specify type of modification and authority)					
E. IMPORTANT: Contractor ☐ is not, ☒ is required to sign this document and return _____ copies to the issuing office.						
14. DESCRIPTION OF AMENDMENT/MODIFICATION (Organized by UCF section headings, including solicitation/contract subject matter where feasible.) THE PURPOSE OF THIS MODIFICATION IS TO: a. EXTEND THE CLOSING OF THIS RFP FROM MAY 12, 2026 TO MAY 15, 2026 AT 10AM. b. PROVIDE RESPONSE TO ALL SUBMITTED QUESTIONS c. PROVIDE REVISED PERFORMANCE WORK STATEMENT SEE ATTACHED CONTINUATION PAGE						
Except as provided herein, all terms and conditions of the document referenced in Item 9A or 10A, as heretofore changed, remains unchanged and in full force and effect.						
15A. NAME AND TITLE OF SIGNER (Type or print)		16A. NAME AND TITLE OF CONTRACTING OFFICER (Type or print) MARICELA BERRONES-GAUER CONTRACTING OFFICER				
15B. CONTRACTOR/OFFEROR (Signature of person authorized to sign)		15C. DATE SIGNED		16B. UNITED STATES OF AMERICA (Signature of Contracting Officer)		16C. DATE SIGNED

The following are sections revised with in the Performance Work Statement. The full document is included with the revisions highlighted in red font.

Section 2:

Responsibility Clarification: Unless otherwise stated, the Contractor is responsible for providing all personnel necessary to deliver contracted primary care services and support clinic operations. When services are identified as 'provided by VA', 'provided by VA via consultation', or similar language, the Contractor is NOT required to staff those positions. However, Contractor SHALL provide space, scheduling, patient support, and coordination to enable those services.

Section 5.6 Specialty Care Services

Space and Equipment Requirements for Telehealth

The Contractor SHALL provide space, patient support, and coordination for telehealth encounters. The Contractor SHALL meet minimum space requirements. VA-provided services may be accommodated within existing or newly configured space without requiring additional dedicated construction unless specified. Contractors shall provide space that meets the standards outlined in the PACT Space Module Design Guide (see section 12). This accommodation requirement applies to all VA-provided personnel supporting services at the site, including but not limited to telehealth technicians, audiology technicians, and other intermittent specialty staff.

Section 12: Space requirements

VA Telehealth Staff Accommodation:

- The Contractor shall accommodate Department of Veterans Affairs (VA) personnel supporting telehealth and related technical services within the clinic. This includes ensuring that sufficient clinical and/or administrative space is available to support the routine and recurring presence of a VA telehealth technician audiology technicians, and other intermittent specialty staff.
- Accommodation shall be achieved primarily through the use of existing clinic space (e.g., telehealth rooms, exam rooms, consultation rooms, or shared work areas) as coordinated between the Contractor and the VA. If necessary to support operational requirements, the Contractor may reconfigure or adjust space within the existing facility.
- This requirement does not mandate the construction of additional space beyond the minimum space requirements outlined in this contract; however, the Contractor shall ensure that space is configured and scheduled in a manner that supports the operational needs of VA telehealth personnel.
- All space utilization shall be coordinated to ensure no disruption to contracted services and must comply with applicable privacy, safety, and infection control requirements.

Section 19: Special Contract Requirements

Contract Start-up Requirements

The contractor shall have (120) calendar days from contract award to commencement of the provision of medical care to VA patients. However, the contractor shall have all start-up requirements in place and ready to commence operation NLT (113) calendar days from contract award. The final seven (7) calendar days will be used for training and resolution of any last minute or unexpected technical or personnel-related challenges.

The following are the answers to the questions submitted:

Question #	Question	Response
1	Pg. 47, Section 5.3. The RFP says, The contractor will provide all medications that are to be administered to patients in the clinic (including vaccines in accordance with current and future Advisory Committee on Immunization Practices (ACIP)/CDC recommendations and guidance. Question: How does the Government expect the contractor to know how to estimate quantities and pricing for future" vaccines or medications that are not part of the current formulary?"	VA provides all medications to the clinic
2	Would the VA confirm that there are no additional power requirements for the network room above, as stated in the network room requirements on page 92 Power requirements: Four (4) L6-20 receptacles One (1) L6-30 receptacle in the closet Each receptacle should be on a separate circuit Receptacles should be located in close proximity to the top or side of the rack, not to impede the walkway or cable management	yes
3	Would the VA confirm if CAT6A wire is required for all Data runs?	yes
4	Will the VA confirm if CAT 6 or CAT6A wire is required for the phone runs?	yes
5	Will the VA confirm if 2 Data and 1 Phone is acceptable at each data/phone location	yes
6	Will the VA confirm if J hooks are acceptable for cable management throughout?	yes
7	Will the VA confirm if the Women's Health Exam room will count as one of the 3 required exam rooms?	VA will provide all medications to be administered in the clinic
8	Will the VA confirm the VA is supplying the primary voice circuit and phone system?	yes

9	Will the VA confirm the proposed facility must meet the on-stage off-stage design?	There is no expectation of clinical pharmacist support from the contractor
10	Will the VA confirm the minimum size for tele health rooms, from the design guide, will be 144 SF and 12 'x12' minimum in size?	The Contractor shall comply with applicable VA design criteria, including space and functional requirements necessary to support telehealth services. While the VA design guides may reference typical room sizes (e.g., approximately 144 SF or 12' x 12'), these values are guidelines and may be adjusted based on final design, layout efficiency, equipment requirements, and VA approval. Final room size and configuration shall be subject to VA review and approval to ensure compliance with operational and clinical needs.
11	Would the VA extend the time until start of services to 180 days and NLT to 173 to allow for the expended time needed to permit and construct a facility? This will allow for a more competitive proposal	Yes, 180
12	Will the VA require the additional exam room listed to also be connected to the PACT area and be on stage off stage?	Per the PWS Space Requirements, the contractor must provide additional exam room space for occasional services, including Women's Health provider, Dentist, Pulmonology technician, and Sleep Study technician; the space may be shared. The PWS does not specifically require this additional exam room to be connected to the PACT area (this can be a shared space).
13	Is a designated office required for the social worker?	A dedicated office for a social worker is not required unless the Contractor proposes to staff such a position onsite. Space planning shall align with the Program for Design (PFD) and support required services and staffing.
14	Is a designated room required for the audiology equipment and technician?	We will need one patient care space and a lab to keep supplies and equipment to do repairs. There will be same day clinic run by the tech and tele audiology assisted by a technician. Audiology has 1 FTE for a clinician at Yreka which we have been willing to share in the past with some of the other specialties
15	Does the VA have a minimum size requirement for the group/conference room?	The clinic shall include conference/group space in accordance with the Program for Design (PFD). Specific square footage is not prescribed; however, the space must be sufficient to support intended functions, including staff collaboration and group activities.
16	Are the Omnicell supply room and the storage closet with room for an Omni Cell the same or separate space requirements?	Same

17	For the occasional services of Women's health provider, Dentist, Pulmonology technician, Sleep Study technician. Does the VA have any specific room requirements such as room size, cabinetry, sink, exam bed?	Specialty and occasional services identified (e.g., Women's Health, Dental, Pulmonology, Sleep Study) are not required to be permanently staffed or dedicated spaces unless explicitly stated in the PWS. The Contractor shall provide flexible, multi-purpose clinical space capable of supporting these services when scheduled. Room configuration (e.g., cabinetry, sink, exam table) shall be appropriate to the intended use and subject to VA review and approval. Dedicated specialty rooms are not required unless specifically identified elsewhere in the contract.
18	For sizing the PACT work area should the PACT work area include space for the 4 Touchdown workstations for the HBPC staff?	Yes
19	How many Women are assigned to the CBOC?	Based on current clinic assignment data, approximately 45 women Veterans are assigned to a Primary Care Provider at the Yreka CBOC. Separately, VSSC projected enrollment data for Siskiyou County (06093) reflects a broader county-level female Veteran enrollee population, increasing from approximately 128 in FY2024 to 177 in FY2033. This projection represents the county-level female Veteran enrollee population and should not be interpreted as the current number of women Veterans assigned to the Yreka CBOC. Offerors should use the current assigned panel figure for clinic staffing assumptions and may use the county-level projection as planning context for potential future demand.
20	Will the VA provide the Thin Prep for Pap Smears?	Per the Women's Health Environment of Care section, the contractor shall provide equipment and supplies necessary to perform Pap tests. The PWS does not identify VA as furnishing Thin Prep/Pap smear supplies.
21	Will the VA provide the names and quantity of the vaccines that have been provided at the CBOC for the last year?	No, but we will supply all meds and vaccines for administration and we will determine needs of clinic

22	Please confirm if the Contractor or the VA will provided the Clinical Pharmacy Services as indicated on page 20 the Contractor provides both the and on page 50 the VA provides both the Clinical Pharmacist Specialist PACT and Clinical Pharmacist Specialist Anti-Coagulation.	Va provides clinical pharmacy services through PACT and pharmany VA processes
23	Please clarify if the Contractor is required to have Xray services on site as indicated on page 26 2.5 Ancillary Support Services Staffing “ Ray Diagnostic Radiologic Technologist: FTE Ratio Performance Standard: 1 FTE technologist per x-ray machine per 8-hour time period with backup required during times of planned and unplanned staff absence”, or will the veterans be referred to the parent VA facility as indicated on page 51 section 5.4 Ancillary “Radiology Services: The contractor is responsible for entering requests for radiology procedures into the VA EHR. All imaging orders shall be clinically appropriate. X-rays shall be performed by VA”	No clinical pharmacy staff are needed to be supplied by the clinic. PACT pharmacists will be available to provide clinical support through consults and telephone visits.
24	Please confirm the VA will provide the GE 5500 with modem EKG machine as indicated on page 51 “Electrocardiogram (ECG) Services: ECGs shall be used that are interfaced with the VA EHR. (The name and model number of the EKG machine needed is GE 5500 with modem. This will be supplied at no cost to the contractor”	Per PWS, GE 5500 EKG machine is furnished by VA at no cost.
25	Podiatry Services – Page 25 Section 2.4 Specialty Care staffing The following shall be provided by the Contractor: “Podiatrist: FTE Ratio Performance Standard: 1.0 FTE per an estimated 950 patients requiring podiatry services with only a single room available for services” however on page 67 5.6 Specialty Care Services - Podiatry Services: Contractor will enter a VA consultation into the EHR – “	Per PWS 5.6 Podiatry Services: Contractor will enter a VA consultation into the EHR. Specialty services such as Podiatry and Physical Therapy SHALL be provided via VA consultation unless explicitly required onsite. The Contractor SHALL support consult entry and coordination.
26	Please confirm if the Contractor is responsible for these services on sight at the CBOC or referrals to the VA.	Per PWS 5.6 Podiatry Services; Podiatry services are requested through VA consultation.

27	Physical Therapy Services - Page 25 Section 2.4 Specialty Care staffing The following shall be provided by the Contractor: “Physical Therapist: FTE Ratio Performance Standard: 1.0 FTE per an estimated 5000 Veterans assigned to the clinic” Physical Therapy Services, however, on page 68 section 5.6 Specialty Care Services Contractor will enter a VA consultation into the HER Please confirm if the Contractor is responsible for these services on sight at the CBOC or referrals to the VA.	Per PWS, Section 5.6 states Physical Therapy Services and Physical Therapy Support Services are handled by VA consultation. Specialty services such as Podiatry and Physical Therapy SHALL be provided via VA consultation unless explicitly required onsite. The Contractor SHALL support consult entry and coordination.
28	Telehealth page 28 section 2.7 Telehealth Services Clinical Staff: Telehealth Clinical Technician (TCT) Position Responsibilities: Provided by VA via consultation. Telepresenter Position Responsibilities: Provided by VA via consultation ; However on page 68 refers to the Contractors staff “Telehealth Authorities of Contracted Staff: It is the responsibility of the contractor to ensure that telehealth services provided by contractor’s staff meet all clinical, technology, business, regulatory and legal requirements. Not all clinical, technology, business, regulatory and legal aspects of telehealth that apply to VA and VA practitioners will apply to contractor’s staff.” Please confirm if the Contractor is responsible for providing a Tele Health Technician (TCT)?	The contractor shall support telehealth services delivered at the site; however, telehealth clinical services, telehealth presenters, and telehealth technicians shall be provided by the Department of Veterans Affairs (VA). The contractor shall be responsible for facilitating telehealth encounters by ensuring appropriate space, scheduling coordination, and patient readiness.
29	QASP Measure “New PC Appointments Completed within 20 Days” The Standard is states 57%, but the acceptable quality level is 76%. Which performance standard will we be held to? Measurement is noted as being VA’s SAIL report, and reported quarterly. Is there a more frequently updated data source that will be provided to ensure we able to measure our performance?	Yes, the quality would be 76%; Access Data source: https://vawww.pbi.cdw.va.gov/pbi_rs/report/GPE/PCS_PACTProfile/SSRS/PACT%20Profile/PACT%20Profile%20-%20Summary (PACT Profile Summary)
30	QASP Measure “Established PC Appointment Wait time” The measure is noted to use a VA scheduling report to and will be measured “periodically.” Is there a	Yes, the quality would be 76%; Access Data source: https://vawww.pbi.cdw.va.gov/pbi_rs/report/GPE/PCS_PACTProfile/SSRS/PACT%20Profile/PACT%20Profile%20-%20Summary (PACT Profile Summary)

specific name of this report and frequency of data availability.

31	QASP Measure PCMHI Penetration This is structured opposite of the New patient measure, where the standard has a higher performance expectation than the acceptable quality level. Which standard will we be expected to meet?	PACT 21 measure is acceptable: https://reports.vssc.med.va.gov/ReportServer/Pages/ReportViewer.aspx?%2fPC%2fPACTCompassCubeSSRS%2fMainMenu&rs%3aCommand=Render
32	QASP Measure SHEP Access Composite Which performance standard will we be held to, the standard or acceptable are different?	SHEP, Access for new and established, Bookability, DamBuster, Next Three Consecutive appt availability; VSSC report in PACT Compass: https://reports.vssc.med.va.gov/ReportServer/Pages/ReportViewer.aspx?%2fPC%2fPACTCompassCubeSSRS%2fMainMenu&rs%3aCommand=Render
33	QASP Measure: BH Composite This measure was dropped from eQM in 2026. Where will this data be provided to contractors from?	The BH Composite measure is no longer tracked through eQM. Performance data for applicable behavioral health measures will be derived from VA enterprise reporting systems, including SAIL and/or other VA-approved data sources. The Government will identify the specific data source(s), metric definitions, and reporting frequency prior to contract performance.
34	QASP Measure: Percent of Teams with Core Teamlet Staffing Ratio 3 or More (PACT 22) The performance standard is set to 100%, which is not attainable if a single PACT fte turnover. Can VA revisit this performance expectation to ensure contractors are not being positioned to receive a CDR for a single employee departing. For reference, the soliciting VA <medical center is not meeting this measure at the time of this question.	NorCal is at 36%, but we do expect 100% as their HR process is different and more flexible than VHA. Can't compare private contractor employment vs. VHA HR process.
35	QASP Measure: Ratio of Non-Traditional Encounters Please clarify the performance target given the wide variation between the standard and AQL.	VVC and Tele-phone appt. are tracked by Access Data: https://app.powerbigov.us/groups/me/apps/057a6693-7190-4fd4-9491-f8a038913689/reports/c0f1e46c-72be-41ec-be41-bf510e4af7a9/ReportSectionca654803494e959a11dc?ctid=e95f1b23-abaf-45ee-821d-b7ab251ab3bf
36	Is the Yreka CBOC required to be exclusively devoted to veteran patients?	The clinic shall be operated for the purpose of providing care to eligible Veterans under this contract. Use of the facility for non-VA patients is not authorized unless explicitly approved by the VA.
37	Please clarify the clinical pharmacy services requirement. Section 2.3 states that discipline-specific PACT team members are to be provided by the contractor; however, page 50 states: "Clinical Pharmacy Services will be	No clinical pharmacy staff are needed to be supplied by the clinic. PACT pharmacists will be available to provide clinical support through consults and telephone visits.

	provided by VA via consultation or telephone.	
38	Please confirm whether the contractor is required to provide one (1) FTE Registered Dietitian per five (5) PACT teams. Additionally, are these services required to be provided on-site, or may they be delivered via telehealth?	PWS indicates VA provides telehealth equipment and support via consultation. Yes, could be delivered by Telehealth. 1:5 ratio is acceptable for Registered Dietitian.
39	Please clarify the requirements for Primary Care Mental Health Integration (PCMHI) staffing and services.	Primary Care Mental Health Integration (PC-MHI) services will be provided through a coordinated model between the Contractor and the Department of Veterans Affairs (VA). The Contractor shall perform initial mental health screening, assessment, and coordination of care within the primary care setting. Ongoing mental health services, including PC-MHI clinical services, will be provided by VA personnel through on-site services, tele-mental health, or referral as appropriate. The Contractor shall support these services through scheduling, space, and care coordination but is not responsible for staffing PC-MHI clinical positions unless otherwise specified.
40	Please clarify the podiatry requirement. Section 2.4 indicates the contractor is to provide podiatry services, while Section 5.6 states: "Podiatry Services: Contractor will enter a VA consultation into the EHR."	Podiatry Services are handled by VA consultation.
41	Please clarify the physical therapy staffing requirement. Section 2.4 indicates the contractor is to provide physical therapy services, while Section 5.6 states: "Physical Therapy Services: Contractor will enter a VA consultation into the EHR."	Physical Therapy Services are handled by VA consultation.
42	Is on-site radiology required, or will imaging services be provided by the VA?	Per PWS Section 5.4, the contractor is responsible for entering radiology orders into the VA EHR. X-rays shall be performed by VA, and other radiology services such as CT, MRI, ultrasound, and mammography are provided by VA via radiology consults.
43	Please clarify the required business hours. Section 3.1 states "7:30 AM to 5:30 PM for regularly scheduled appointments and walk-ins." Given the anticipated patient volume, would 7:30 AM to 4:30 PM be acceptable? Additionally, please confirm that business hours are Monday through Friday.	RFP requires at least 8 hours between 7:30 AM–5:30 PM, Monday–Friday. It is acceptable to have 7:30 AM to 4:30 PM, M-F.

44	Is an on-stage/off-stage clinic design required?	An on-stage/off-stage clinic design model is not required under this contract. The Contractor may propose any efficient and functional layout that supports patient care delivery, staff workflow, and privacy requirements, subject to VA review and approval.
45	Page 163 states: "Provide State Department of Health and Human Services license to operate a clinic, be accredited by the Joint Commission, and possess a State Controlled Substance Certificate for the State of California." a. Is Joint Commission accreditation required at the time of proposal submission? b. Are state licensure requirements expected to be in place prior to award, or may they be obtained post-award?	Required State licenses, certifications, and inspection documentation are not required at the time of proposal submission. Offerors shall demonstrate their capability and plan to obtain all required licenses, certifications, and inspections as part of their proposal. All required licenses, including the State Department of Health and Human Services license, State Controlled Substance Certificate, and applicable State and local inspections (fire, building, life safety), must be obtained and submitted prior to commencement of services, in accordance with Contract Start-Up Requirements.
46	Will the VA provide lab specimen collection supplies, including specimen tubes?	SMC has supplied via an Omnicell in the past - reach out to SMC.
47	Regarding Paragraph 5.2, would the VA consider providing Women's Health supplies, including ThinPrep Pap Test kits, Pap smear collection kits, cytobrushes, and iFOBT kits? In our experience, these items can be challenging for contractors to procure.	VHA should not and would not provide Women's Health Supplies to contractors.
48	Please confirm the final deadline for submission of offeror questions, as no date is identified in the solicitation.	Questions deadline of April 13 was posted on AMENDMENT 001
49	Please extend the due date for 30 days after final questions have been answered	Final deadline was extended to May 12 was posted on AMENDMENT 002
50	Please confirm whether Technical, Administrative, Past Performance, and Price volumes should be submitted as separate files.	Yes they should be submitted in separate files.
51	Please list documents government requires in the administrative volume.	Please refer to page 162 (c) Proposal Package Contents 1 - 5.
52	Please clarify the file size limitation for emails when submitting the proposal	The size of the email is limited to 5 megabytes (MB) but multiple emails are allowable.
53	Please clarify the stated 30-day price hold period the Government's intention.	This allows for the government to make contract award.
54	Please advise whether the Government intends to conduct a price realism analysis	Page 164 states this will be a comparative evaluation of all 3 factors.

in accordance with FAR 15.404-1(d).

55	Please explain the rationale for the maximum contract value identified in the solicitation and whether the Government would consider adjusting the contract ceiling.	This contract will be an IDIQ (indefinite delivery, indefinite quantity) and a maximum ceiling value must be identified. The government does not intend on adjusting the ceiling for this contract.
56	Will the government accept or consider offers that are greater than the contract maximum?	The government will evaluate responses that have submitted all requested items in section c page 162
57	Please provide the current number of enrolled and billable veterans assigned to the Yreka CBOC, including the most recent billable roster.	Per PWS Section 2.1, the total estimated patients assigned to the site is 953. The PWS does not provide a separate current enrolled count, billable roster count, or PMPM billing count.
58	Please provide historical enrollment data and projected enrollment for the base and option periods.	Historical and projected enrollment data are not provided as part of this solicitation. Offerors shall base staffing and operational assumptions on the PWS requirements and standard VA panel size and workload expectations. Final enrollment and panel assignments will be provided post-award. Max panel of 1350 patients, projected enrollment is 150-350 patients from base of 950 patients potentially.
59	Please identify the total number of VA Mission Act primary care referrals generated for Yreka CBOC enrollees during the previous 12 months.	Historical referral volume data is not included in this solicitation. Offerors should base proposals on anticipated patient panel size and typical utilization patterns for similar VA Community-Based Outpatient Clinics (CBOCs).
60	Please identify all VA-staffed positions expected to be onsite at the clinic and the anticipated days per week each position will be present.	The Department of Veterans Affairs (VA) will provide certain clinical and technical personnel in support of services delivered at the clinic, including telehealth staff and intermittent specialty support (e.g., audiology technicians and other consult-based services). Specific staffing positions, frequency of onsite presence, and scheduling will vary based on patient demand, service utilization, and operational requirements, and are not fixed under this contract. The Contractor shall be responsible for accommodating VA-provided personnel and coordinating services but shall not be responsible for staffing these roles.

61	Please clarify which clinical services are to be provided directly by the Contractor versus the VA, including primary care, mental health, dental, physical therapy, radiology, and specialty consult services. Clinical services under this contract are delivered through a hybrid model with defined responsibilities between the Contractor and the Department of Veterans Affairs (VA). Contractor-Provided Services: The Contractor shall provide all primary care services, including Patient Aligned Care Team (PACT) functions, routine clinical care, initial mental health screening, and care coordination. The Contractor is also responsible for all clinic operations, including staffing, administration, and onsite clinical support services required to deliver primary care. VA-Provided Services: The VA will provide certain clinical and technical services, including telehealth services (with VA-provided staff and equipment), specialty care services (e.g., dental, physical therapy, radiology, and specialty consults), and select mental health services beyond initial screening. These services will be delivered either through telehealth or through referral to VA facilities. Shared / Coordinated Services: Mental health services will be delivered through a coordinated model in which the Contractor performs initial screening, triage, and referral, and the VA provides ongoing specialty mental health care as needed. The Contractor shall coordinate care and support VA-provided services but shall not be responsible for staffing these roles unless otherwise specified.
62	Please clarify how mental health services are expected to be delivered, including contractor-provided services, VA-staffed services, tele-mental health, and/or referral to the parent VAMC. Per PWS Section 5.5, the contractor provides primary care-based mental health and substance use screening, triage, brief intervention, same-day response, crisis response, documentation, referral/warm handoff, and coordination. General and Specialty Mental Health staffing is identified in Section 2.6 as provided by VA via consultation.
63	Please identify which vaccines the contractor will be responsible for purchasing and provide historical utilization quantities for the prior 12 months. No vaccines are required to be supplied by contractor.
64	Please confirm whether the successor contractor will inherit existing enrollees' vesting dates and be authorized to bill PMPM rates for up to one year following contract award. Enrollment, vesting, and PMPM billing provisions will be administered in accordance with contract terms and applicable VA policies. Offerors should not assume automatic transfer of incumbent-specific conditions unless explicitly stated in the contract.
65	Please confirm whether podiatry services may be performed in designated procedure rooms. Podiatry services are not required to be provided directly by the Contractor under this contract. If podiatry services are performed onsite by VA personnel, designated procedure or appropriately equipped exam rooms may be utilized, provided all applicable clinical, safety, and equipment requirements are met.

66	Please clarify whether physical therapy services are required under this contract and, if so, provide detailed room size and space requirements.	Physical therapy services are not required to be provided directly by the Contractor under this contract. Such services will be provided by the Department of Veterans Affairs (VA) through referral to VA facilities, community care, and/or telehealth services as appropriate. The Contractor is not required to provide dedicated physical therapy space beyond general clinical space capable of supporting occasional or shared use, as outlined in the Program for Design.
67	Please confirm whether contractors may utilize community-based radiology services in lieu of providing onsite X-ray services.	PWS states X-rays are performed by VA; contractor enters orders into VA EHR.
68	Please confirm the required number of exam rooms, consult rooms, and telehealth rooms per PACT team in accordance with the PACT Space Module Design Guide.	The Contractor shall design the clinic in alignment with applicable VA design criteria, including the PACT Space Module Design Guide, to support the required number of PACT teams. Specific room counts (exam, consult, telehealth) shall be based on the Contractor's proposed design and operational approach, provided that access, throughput, and care delivery requirements are met. Final room counts are subject to VA review and approval and are not limited to a fixed prescriptive number unless explicitly stated.
69	Please confirm whether exam tables are required within telehealth rooms.	PWS indicates VA provides telehealth equipment and support via consultation. Exam tables are not required within telehealth rooms unless the space is intended to serve a dual clinical function. Telehealth rooms shall meet functional requirements outlined in the PWS and applicable VA design guidance.
70	Please confirm the required number, type, and square footage of conference, group, and multi-purpose specialty rooms.	The required number and type of conference, group, and multi-purpose specialty rooms shall align with the Program for Design (PFD). Specific square footage is not prescribed beyond the PFD; however, such spaces must be appropriately sized and configured to support intended functions, including staff collaboration, patient education, group activities, and intermittent specialty use. Multi-purpose use of these spaces is acceptable, provided the design supports operational efficiency, privacy, and safety requirements.

71	Please confirm the required number of procedure rooms and women's health exam rooms and whether a single room may satisfy both requirements.	Per the PWS Space Requirements, the minimum space includes one women's health exam/lactation room with adjacent restroom and one specialty exam room. The clinic shall meet all exam room and specialty space requirements in accordance with the Program for Design (PFD). A dedicated women's health exam room is required and must meet all applicable privacy, equipment, and adjacency requirements. Procedure capabilities may be accommodated within appropriately equipped exam rooms; however, combining functions into a single room is acceptable only if all clinical, safety, and operational requirements for each use are fully met.
72	Please clarify requirements for shared specialty use of rooms (e.g., dentistry, pulmonology, sleep study) and required square footage to support multi-use.	Specialty services that are not routinely provided onsite may be supported through shared or multi-purpose clinical space. Rooms intended for shared specialty use (e.g., dentistry, pulmonology, sleep-related services) shall be designed to accommodate required equipment, workflows, and safety standards for each intended use. Specific square footage is not prescribed beyond the Program for Design (PFD); however, the Contractor shall ensure sufficient space, infrastructure, and flexibility to support safe and effective multi-use operations.
73	Please confirm whether the clinic must be designed using an Onstage/Offstage layout.	The PWS requires the contractor's space to meet the PACT Space Module Design Guide and states the contractor's floor plan shall meet that design standard. The PWS does not separately use the term "Onstage/Offstage," so this requirement should be read through the PACT Space Module Design Guide. An Onstage/Offstage layout is not required; however, the proposed design must demonstrate efficient workflow, patient privacy, staff collaboration, and operational effectiveness consistent with PACT principles. Final design will be subject to VA review and approval
74	Please advise whether the incumbent clinic meets the space, layout, and building requirements of the solicitation.	The Government does not make representations regarding the compliance of any existing facility with the requirements of this solicitation. Offerors are responsible for proposing facilities that meet all stated requirements.
75	If the incumbent clinic does not meet current requirements, please confirm whether it may be operated on a grandfathered or 'as-is' basis in the next contract.	All proposed facilities must meet the requirements of this solicitation. Operation under a grandfathered or "as-is" condition is not permitted unless explicitly approved by the VA.

76	Please clarify furnishing responsibilities for FF&E, including responsibility for specialty equipment such as dental chairs, physical therapy equipment, radiology equipment, and Omnicell units.	The Contractor shall be responsible for providing all furniture, fixtures, and equipment (FF&E) necessary to support required primary care services under this contract. Specialty equipment associated with services not required to be provided by the Contractor (e.g., dental, physical therapy, radiology) is not required. The Department of Veterans Affairs (VA) will provide Government-furnished equipment (GFE) only where explicitly identified, including select IT systems and equipment necessary for VA connectivity and operations. All other equipment required for contract performance shall be the responsibility of the Contractor.
77	Please confirm whether the VA will furnish EKG equipment at no cost to the contractor.	Per PWS, GE 5500 EKG machine is furnished by VA at no cost.
78	Please confirm whether the VA will furnish clinic telephones and whether existing telephone numbers will transfer to the successor contractor.	yes
79	Please confirm IT infrastructure requirements, including CAT6 vs. CAT6A cabling standards.	yes
80	Please clarify physical security, IT security, and privacy requirements for the facility and confirm whether these must be demonstrated at proposal submission or post-award.	The Contractor shall comply with all applicable VA requirements for physical security, information security, and privacy, including but not limited to VA directives, handbooks, and federal regulations (e.g., HIPAA, FISMA). At the time of proposal submission, the Contractor shall describe their approach and capability to meet these requirements. Full implementation, validation, and compliance documentation (including physical security measures, IT systems accreditation, and privacy controls) shall be required post-award and prior to the start of clinical operations, subject to VA review and approval.
81	Please clarify the requirement to submit a State Department of Health and Human Services license, including the regulatory basis and timing of submission.	A State Department of Health and Human Services license is required for contract performance. This license is not required at the time of proposal submission. The Contractor shall demonstrate the ability to obtain the required license in their proposal and must obtain and provide the license prior to commencement of services in accordance with Contract Start-Up Requirements.
82	Please clarify the requirement for a State Controlled Substance Certificate and confirm whether individual provider DEA registration satisfies this requirement.	The Contractor shall ensure all providers meet applicable federal and state controlled substance requirements, including DEA registration and any required State Controlled Substance Certificate. Proof of capability shall be included at proposal submission. Required licenses and registrations must be in place prior to the start of clinical services.

83	Please clarify whether Joint Commission accreditation is required of the Contractor entity or whether VA facility accreditation satisfies this requirement.	Joint Commission accreditation is not required of the Contractor entity. VA facility accreditation satisfies this requirement.
84	Please confirm whether proof of BLS/ACLS certifications and official academic transcripts for proposed staff are required at proposal submission or may be submitted post-award.	Full implementation, validation, and compliance documentation (including physical security measures, IT systems accreditation, and privacy controls) shall be required post-award and prior to the start of clinical operations, subject to VA review and approval.
85	Please confirm whether copies of state and local facility inspections for fire, building, and life-safety codes may be submitted prior to clinic opening instead of with the proposal.	The Contractor shall comply with all applicable fire, building, and life safety codes and requirements as established by the Authority Having Jurisdiction (AHJ) and VA standards. Documentation of required inspections and approvals is not required at the time of proposal submission but must be obtained and submitted prior to clinic opening and commencement of services. All facilities will be subject to VA review and approval to ensure compliance with applicable safety and operational requirements.
86	Please confirm whether a current VETS-4212 report is required at proposal submission and whether submission of the most recently filed annual report is acceptable.	Will be required by time of contract award
87	Please advise whether the Government intends to conduct site visits prior to award and the anticipated notice period.	The Government may conduct a pre-award site visit at its discretion. If scheduled, details and notice will be provided to all prospective Offerors via formal amendment to ensure equal access.
88	Please extend the clinic startup period from 90 days to 180 days due to permitting and construction timelines.	The Government intends to extend the proposal submission timeline for the purpose of obtaining licenses or certifications from 90 days to 120. Offerors are expected to demonstrate capability and a clear plan to obtain required licenses and certifications within the post-award period.
89	Can VA provide insight on whether a full-time Clinic Manager required and if so, would the VA prefer they be a Registered Nurse?	Clinic Manager is preferred, but is not required. The Contractor shall provide clinic management and administrative oversight sufficient to meet PWS operational requirements. The solicitation does not mandate a specific title (e.g., Clinic Manager) or require that the role be filled by a Registered Nurse.
90	The solicitation does not state that the contractor is responsible for providing a 1.0 FTE Clinic Administrator. Does the VA require a 1.0 FTE Clinic Administrator? If so, does this administrator need to be a Registered Nurse?	Clinical Administrator is usually a provider or RN. The PWS does not prescribe a requirement for a 1.0 FTE Clinic Administrator. The Contractor is responsible for providing appropriate administrative staffing to ensure clinic operations meet all performance and compliance requirements. No requirement is specified that such personnel be a Registered Nurse.

91	Can the VA confirm that when the Physician serves as the Clinic Medical Director, their panel is reduced by 0.05 per PACT Team?	The Physician Director role is defined as 0.05 FTE per PACT team. Panel sizes and workload expectations are determined by VA in accordance with PCMM standards. Offerors shall propose staffing models that support both clinical care and leadership responsibilities.
92	Please provide the FTE requirement for the contractor-provided phlebotomist.	We have no plans to provide an FTE or contractor
93	The solicitation states that the General and Specialty MH Staffing will be provided by VA but that the contractor will provide scheduling personnel. Please confirm that the General and Specialty MH Staff will be provided by VA and that the contractor will provide on-site scheduling personnel for the VA-staffed General and Specialty MH staff.	General and Specialty Mental Health (MH) clinical services will be provided by Department of Veterans Affairs (VA) personnel. The Contractor shall provide on-site administrative and scheduling support necessary to coordinate care, including scheduling for VA-provided MH services where applicable. The Contractor shall not be responsible for staffing General or Specialty Mental Health clinical positions unless otherwise specified in this contract.
94	The RFP states, a copy of licenses shall be provided with offer and will be updated annually. However, in the Instructions to Offeror, there is no mention of providing the staff or licenses. Is the contractor to provide the proposed staff and their licenses with the proposal submission? And if so, in what section should this be provided?	Per PWS Section 2.8, a copy of licenses shall be provided with the offer and updated annually. Any changes related to provider licensing or credentials must be reported immediately to the VA Credentialing Office.
95	This section states that regularly scheduled appointments shall be from 7:30am - 5:30pm. However, the previous section states that "services shall be available from the Contractor for a period of at least eight hours between the hours of 7:30am and 5:30pm Monday through Friday. Please clarify the required business hours for this CBOC. Is an 8:00am - 4:30pm schedule acceptable?"	RFP requires at least 8 hours between 7:30 AM–5:30 PM, Monday–Friday building hours. Patient Schedule Hours are 8:00 am -4:30 pm is acceptable.

96	The RFP states: : The contractor will provide all medications that are to be administered to patients in the clinic (including vaccines in accordance with current and future Advisory Committee on Immunization Practices (ACIP)/CDC recommendations and guidance). Will the VA provide a complete list of medications and vaccines to include all injectable medications that the Contractor is expected to provide? And, will the VA provide historical monthly and annual usage at the Yreka CBOC for the past 3 years?	VA will provide all medications to be administered in the clinic
97	The solicitation states that the contractor will provide all medications that are to be administered to patients in the clinic. Please provide a list of all medication and vaccine types delivered/administered, along with quantities for the most recent calendar year.	No, VA will provide all medications to be administed in the clinic
98	The solicitation states that clinical pharmacy sevicees "will be provided by VA consultation or telephone. These services shall be provided by a clinical pharmacist practitioner". However, in section 2.3 about Discipline Specific PACT Team Members, it states that a Clinical Pharmacist Practitioner shall be provided by the contractor. Please clarify whether the VA or the contractor will provide the Clinical Pharmacist Practitioner.	There is no expection of clinical pharmacist support from the contractor
99	The solicitation sates that "X-rays shall be performed by VA". However, in section 2.5 it states that the contract shall provide 1 FTE technologist with a backup required. Please clarify whether the VA or the contractor will provide the x-ray diagnostic radiologic technologist.	PWS states X-rays are performed by VA; contractor enters orders into VA EHR.
100	The solicitation states that Anticoagulation services will be provided by the VA. However in section 2.3 it states that the contractor shall provide a Clinical Pharmacist Practitioner for Anti-Coagulation Please clarify whether the VA or the contractor will provide	The VA will manage all patients on anticoagulation medications through a centralized clinic that is run telephonically.

Anticoagulation services.

101	The contractor shall provide same day MH access and same day MH evaluation. Any Veteran reporting or identified as being in crisis (including suicidality), will receive an immediate crisis response. Services shall be provided as follows: • Veterans New to MH: In Person: Any Veteran new to MH requesting or referred for care in person will be seen in person the same day by a Licensed Independent Provider (LIP) to screen for and address immediate care needs. By Phone: Any Veteran new to MH calling to initiate care will be scheduled for an initial evaluation. Schedulers answering the phone will ask if the Veteran needs to speak with a provider immediately. If an urgent request is made or suggested, an immediate crisis response will be initiated and follow-up care will be provided, as needed. If an urgent response is not indicated, a LIP will call the Veteran back the same day or no later than the next calendar day. Per staffing guidelines, VA is providing all Mental Health staff. Will the VA please clarify the contractor's role and responsibility relate to general and specialty mental health?	The Contractor shall provide same-day access to care, including initial screening, triage, and coordination for Veterans presenting with mental health needs, in accordance with PWS requirements. General and Specialty Mental Health clinical services will be provided by Department of Veterans Affairs (VA) personnel. The Contractor is not responsible for staffing Mental Health clinical providers unless otherwise specified. The Contractor shall ensure that Veterans identified as needing mental health services are appropriately assessed, scheduled, and connected to VA-provided services, including same-day access when clinically indicated. In situations involving urgent or emergent mental health needs, the Contractor shall initiate immediate response protocols and coordinate transfer or handoff to VA or appropriate emergency services as required. The Contractor's role includes screening, intake, scheduling, care coordination, and support functions necessary to facilitate timely access to VA-provided Mental Health services.
102	The solicitation states that for podiatry and physical therapy services, the contractor will enter a VA consultation into the EHR. However in section 2.4 it states that the contractor shall provide a podiatrist and physical therapist. Please clarify wheter the VA or the contractor will provide a podiatrist and physical therapist.	Podiatry and Physical Therapy services will be provided by the VA through consultation and referral processes. The Contractor is not required to provide on-site podiatry or physical therapy staff. The Contractor's responsibility is limited to initiating appropriate consults within the VA system in accordance with PWS requirements.

103	Can the VA confirm that it intends to provide all telehealth equipment and staff. The RFP states that the Contractor shall make provisions for toll free telephone care, twenty four (24) hours a day, seven (7) days a week, including evenings, weekends, and holidays for all Veterans, in accordance with VHA Directive 1090, Telephone Access for Clinical Care. Can the Contractors use the VA's after care line to satisfy this requirement?	VA will provide telehealth platforms, equipment, and associated clinical staff as applicable for VA-delivered services. The Contractor is responsible for supporting telehealth coordination and ensuring patients are appropriately scheduled and prepared for telehealth encounters. In accordance with VHA Directive 1090, the Contractor shall ensure 24/7 telephone access for Veterans. This requirement may be satisfied through the Contractor's own system or through coordination with VA-approved after-hours nurse triage or call center services, subject to VA approval.
104	Can VA confirm whether it will provide the phone circuit and the physical telephone equipment	yes
105	The solicitation states that the contractor shall provide an Omnicell supply room and a storage closet with space for an Omnicell. Please confirm that the VA intends to provide the Omnicell unit and that both spaces described above are needed. If confirmed, please provide the space requirements for the storage closet that will house the Omnicell (ie. temperature, dimensions, controlled access, etc.).	VA will provide the Omnicell.
106	The solicitation states that the contractor shall provide a Clinical Pharmacist Practitioner, however, the space requirements do not state that the contractor is required to provide a dedicated space for this staff member. Please provide the space requirements for the contractor-provided Clinical Pharmacist Practitioner.	They will use the 4-touch down spaces indicated in Question #18: SW, CPP, Dietary/Nutrition, PCMH
107	The solicitation states that the contractor shall provide 4 touchdown workstations for HBPC. What are the space requirements for these touchdown workstations?	Per the PWS Space Requirements, four touchdown workstations for HBPC are required. The PWS does not further specify each touchdown workstation user or function in that section.
108	The solicitation states that the contractor shall provide a Dietician/Nutritionist, however the space requirements do not state that the contractor is required to provide a dedicated space for this staff member. Please provide the space requirements for the contractor-provided Dietician/Nutritionist.	As stated in Questino #18, #106, 4 touch down spaces will be used by Dietician/Nutritionist.

109	Does the current CBOC meet the PACT Space Module Design Guide. If not will it be grandfathered in?	The contractor's facility shall meet the requirements of the PACT Space Module Design Guide and associated VA design standards. The Government does not provide representations regarding the compliance of any existing facility. All proposed space must meet solicitation requirements. Grandfathering is not applicable unless explicitly approved by the VA.
110	Will pharmacy services be available at the Clinic? If so, will it be thru Tele-Pharmacy or clinical Pharmacist? If so will a Tele-Pharmacy room be needed or private office?	All VA pharmacy services will be accessed through e-consult or telephone visits with patients. No space is needed for pharmacy clinical support.
111	The RFP states under "Operating and Maintenance (O&M) Plan, that the Contractor shall have an O&M plan which shall describe the organization and structure the workforce of both operations and maintenance personnel. Can the VA clarify whether they would like this included in the proposal submission, and if so, in which volume or section of the technical submission?	This should be included in subfactor c.
112	The RFP states, Contractor shall meet VHA standards regarding EOC and shall provide EOC management plans addressing safety, security, hazardous materials, hazardous waste, emergency preparedness, life safety, medical equipment, infection control, and utility systems with the submission of their proposal and within 15 calendar days after contract award. However, nowhere in the Instructions to Offeror, does it mention providing EOC management plans. Where in the technical proposal would the VA like to see the EOC management plans? Or can this be submitted post-award?	Offerors shall demonstrate their approach and capability to meet Environment of Care (EOC) requirements within the technical proposal. Full EOC management plans are not required at proposal submission but shall be developed, submitted, and approved within 15 calendar days after contract award and prior to commencement of services.
113	Could the VA please confirm the Power Requirements and if two 20-amp dedicated receptacles are acceptable?	The Contractor shall ensure that all facility systems meet applicable federal, state, and local codes and standards. Two (2) 20-amp dedicated receptacles are generally acceptable for standard clinical operations; however, final electrical requirements must be validated based on the specific equipment and clinical services proposed.
114	The solicitation states that the contractor shall have (90) calendar days from contract award to commencement of the provision of medical care to VA patients. Can this duration be extended to 120 days to allow adequate time for VA credentialing?	The Government intends to extend the proposal submission timeline for the purpose of obtaining licenses or certifications from 90 days to 120. Offerors are expected to demonstrate capability and a clear plan to obtain required licenses and certifications within the post-award period.

115	The RFP states that the Contractor must provide a State Department of Health and Human Services license to operate a clinic, possess a State Controlled Substance Certificate for the State of California and provide a copy of the latest State or local inspection of the facility that substantiates appropriate codes for building, fire, and safety are met. Is the Contractor required to provide this licenses, certificate, and copy of the inspection at proposal submission or post-award?	Required State licenses, certifications, and inspection documentation are not required at the time of proposal submission. Offerors shall demonstrate their capability and plan to obtain all required licenses, certifications, and inspections as part of their proposal. All required licenses, including the State Department of Health and Human Services license, State Controlled Substance Certificate, and applicable State and local inspections (fire, building, life safety), must be obtained and submitted prior to commencement of services, in accordance with Contract Start-Up Requirements.
116	The solicitation identifies Past Performance as Factor 2. However, Subfactor A under Factor 1 instructs contractors to describe their experience providing services in a private hospital, office, or clinic environment within the past three years. Could you please clarify whether this information related to past performance should be included under Subfactor A of Factor 1, or whether it should instead be submitted as part of the separate Past Performance volume associated with Factor 2?	This information should be provided under Factor 1. Subfactor A is asking vendors to describe experience, how quality, and performance measures will be met. Factor 2 is asking vendor to identify 3 contracts with detailed information. Please review both sections.
117	The RFP lists out Subfactor A-D under Factor 1, Past Performance under Factor 2, and Price under Factor 3. Would the VA like Contractors to submit three separate Volumes for each Factor?	Yes they should be submitted in separate files.
118	Under "Proposal Package Contents," the RFP states we should include SF1449, the Acknowledgement of all amendments, Completed Attachments in Section D, and a Copy of VETS 4212 Report. Can the VA confirm where in the proposal they would like these documents included?	These documents can be submitted as individual pdf documents along with the proposal package.
119	The RFP states under Subfactor A-Quality Standards of Care that the Contractor should "describe your experience in providing services in a private hospital, office, or clinic environment within the past 3 years." Can the VA confirm whether they want the Past Performance listed in Factor 2 to be included under Subfactor A or in a	Please review both factors Factor 1 Subfactor A is requesting detailed experience and how quality and performance measures will be met. Factor 2 is asking for 3 contracts for past performance with detailed contract information. Review each factor for the specific details that are requested.

	separate Volume?	
120	Could the VA please confirm the maximum email attachment size (in MB) for submissions?	The size of the email is limited to 5 megabytes (MB) but multiple emails are allowable.
121	Can the VA confirm if there is a font size and number that Contractors should adhere to?	no specific font/size
122	The RFP states the Contractor must provide a State Department of Health and Human Services license to operate a clinic and possess a State Controlled Substance Certificate for the State of California.	Required licenses, certifications, and inspections (including State Department of Health and Human Services license and State Controlled Substance Certificate) are not required at the time of proposal submission. The Contractor shall demonstrate the ability to obtain all required licenses and certifications as part of their proposal. All required licenses, certifications, and inspection documentation must be obtained and submitted prior to commencement of services, within a timeframe specified post-award.
123	To promote adequate competition and given the SDVOSB nature of the requirement and timeline to secure these items with the State of California will the government please allow a major subcontractor to provide these items?	Please reference VAAR clause 852.219-75 on page 153 of the RFP.
124	Alternatively would the government please extend the delivery timeline to have these items be completed prior to the commencement of the contractor's treatment of VA patients as noted in Contract Start-Up Requirements on page 105?	The Government does not intend to extend the proposal submission timeline for the purpose of obtaining licenses or certifications. Offerors are expected to demonstrate capability and a clear plan to obtain required licenses and certifications within the post-award period.

125

The RFP states the Contractor must provide a copy of the latest State or local inspection of the facility that substantiates appropriate codes for building, fire, and safety are met.

Per the PWS Space Requirements, VA will inspect the contractor's facility before contract start and may inspect throughout the period of performance. The contractor must comply with NFPA life safety, ADA, applicable codes, and VA-identified corrections before clinic opening. The PWS does not expressly require submission of the latest state/local inspection reports with the proposal.

Copies of required State and local inspections (fire, building, life safety) are not required at proposal submission.

The Contractor shall provide documentation demonstrating compliance with applicable codes prior to clinic opening.